

Visolone Tablet 1mg

Ingredient(s):

Each tablet contains:

Prednisolone.....1mg

Pharmacodynamics:

1. Prednisolone can decrease or prevent tissue responses to inflammatory processes, thereby reducing development of symptoms of inflammatory without affecting the underlying cause. Prednisolone inhibits accumulation of inflammatory cells including macrophages and leukocytes at sites of inflammation. It also inhibits phagocytosis, lysosomal enzyme release, and synthesis and/or release several chemical mediators of inflammation.

Pharmacokinetics:

1. Prednisolone is readily absorbed from the gastrointestinal tract and already exists in a metabolically active form; biological half-life is about 60 minutes.
2. Peak plasma concentration obtained 1 or 2 hours after oral administration. It has a usual plasma half-life of 2 to 3 hours and is excreted in the urine as free and conjugated metabolites. Prednisolone crosses the placenta and small amounts are excreted in the breast milk.

Indication(s):

For the treatment of bronchial asthma, allergic disease, rheumatoid arthritis, rheumatosis, Still's disease, rheumatoid myocarditis, psoriasis, osteoarthritis, rheumatic myelitis, gouty arthritis, acute and chronic gout and other inflammatory skin disease.

Dosage and Administration:

Oral: The tablets should be swallowed with a drink of water.

Adult: 20-40mg daily (acute conditions up to 80mg daily) reducing gradually to maintenance level when symptoms have subsided. Maintenance dosage is usually 5-20mg daily reached in about two weeks by reduction of the daily dosage by 5mg or 2.5mg, two or three times a week.

Children: Fractions of adult dosage may be used (eg 75% at 12 years, 50% at 7 years and 25% at 1 year), but clinical factors must be taken into consideration.

Contraindication(s):

Prednisolone is contraindication in patient with systemic fungal infections and those who have shown hypersensitivity to any component of this product.

Precaution(s) / Warning(s):

1. The adverse effects of Prednisolone are nearly always due to its use in excess of normal physiological requirement. They should be treated symptomatically, where possible, the dosage being reduced or the drug slowly withdrawn.
2. Prednisolone may mask some signs of infections and new infections may appear during its use. There may be decreased resistance and inability to localize infection when Prednisolone is used.
3. Because of possibility of fluid retention, care must be taken when corticosteroids are administered to patients with congestive heart failure. Corticosteroids may worsen diabetes mellitus, osteoporosis, hypertension, glaucoma and epilepsy.
4. Care should be taken when there is a history of severe affective disorders (esp. severe history of steroid psychosis), previous steroid myopathy or peptic ulceration.
5. **Scleroderma renal crisis.**
Caution is required in patients with systemic sclerosis because of an increased incidence of (possibly fatal) scleroderma renal crisis with hypertension and decrease urinary output observed with a daily dose of 15mg or more prednisolone.
6. Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.

Pregnancy and Lactation:

The use of Prednisolone in pregnancy, nursing mothers, or women of childbearing potential requires that the possible benefits of drugs be weighed against the potential hazards, to the mother and embryo or fetus.

Drug Interactions:

Potentially hazardous interactions

1. **Anti-convulsant drugs:** The actions of Prednisolone are blunted in patients receiving concomitant therapy.
2. **Rifampicin:** Non-responsiveness of Prednisolone has been attributed to concurrently administered Rifampicin which is a known inducer of drug metabolism.

Other significant interactions

1. **Cyclosporine:** Cyclosporine decreases the metabolism of Prednisolone in the early phase of the combination therapy. Accordingly, plasma half lives are prolonged and clearance reduced. Prednisolone dosage in such cases may, therefore, require adjustment.
2. **Corticosteroids:** Previous long-term treatment with other corticosteroids may reduce plasma Prednisolone half-life.
3. **Non-steroidal anti-inflammatory drugs (NSAIDs):** Concurrent administration of corticosteroids and large doses of aspirin may cause changes in salicylate plasma protein binding and its rate of metabolism, thereby lowering plasma salicylate levels.
4. **Antacids:** Prednisolone absorption may be impaired if a small amount of it is taken with large doses of antacids.
5. **Insulin:** Prednisolone may also decrease the effects of insulin by raising the level of blood sugar. As a result, the requirement of insulin is generally increase.
6. **CYP3A inhibitors:** Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side effects. The combination should be avoided unless the

benefit outweighs the increased risk of systemic corticosteroid side effects, in which case patients should be monitored for systemic corticosteroid side effects.

Effects on Ability to Drive and Use Machines:

If insufficient sleep occurs, the likelihood of impaired alertness may be increased, patients should make sure they are not affected before driving or operating machinery.

Side Effect(s) / Adverse Reaction(s):

The prolonged therapy may result in suppression of pituitary adrenal function and the principal complications resulting from prolonged therapy with Prednisolone are fluid and electrolyte disturbances, hyperglycemia and glycosuria; increased susceptibility to infections, including tuberculosis; peptic ulcers which may bleed or perforate; osteoporosis; a characteristic myopathy; behavioral disturbances; posterior subcapsular cataracts; arrest of growth; and Cushing’s habitus, consisting of “moon face”, “buffalo hump”, enlargement of supraclavicular fat pads, “central obesity”, striae, ecchymoses, acne and hirsutism.

Symptoms and Treatment of Overdose, and Antidote(s):

There is no specific antidote for it’s over dosage; therefore, management of the patients should consist of symptomatic and supportive therapy.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from direct sunlight, heat and moisture. Always keep in its original packaging provided.

Product Description & Packing(s):

A white color tablet, one side impressed with a mark of “Y”.



Blister packing of 10’s x 100

Manufacturer and Product Registration Holder:

Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)

Lot 3, 5, 7, 12 & 14, Jalan P/7, Section 13,

Kawasan Perindustrian Bandar Baru Bangi,

43000 Kajang, Selangor, Malaysia.

Ordering Line: 1 800 88 3027

Product Info: 1 800 88 3679